

Pellet Therapy Dosing Guide

Testosterone and estradiol pellet dosing, insertion technique, monitoring, and complication management Danielle Alcalá · nopriorauthorization.com

PELLET DOSING – FEMALES

Patient Profile	Testosterone Dose	Estradiol Dose	Re-pellet Interval	Notes
Pre-menopausal – low libido/energy	50–100mg T	0–6mg E2 (optional)	3–4 months	Start low – androgenic side effects more common in pre-menopausal women
Peri-menopausal	75–150mg T	6–12.5mg E2	3–4 months	Titrate based on symptom response and lab levels at 4–6 weeks post-pellet
Post-menopausal – mild symptoms	100–150mg T	12.5–18.75mg E2	3–4 months	Most common dosing range for post-menopausal patients
Post-menopausal – significant symptoms	150–200mg T	18.75–25mg E2	3–4 months	Higher end dosing for active women or inadequate response at lower doses
Very active / athletic women	150–225mg T	Per symptoms	3–4 months	Higher T clearance in very active patients – may need higher dose or shorter interval

PELLET DOSING – MALES

Patient Profile	Testosterone Dose	Re-pellet Interval	Notes
Mild hypogonadism (T 300–400)	600–800mg T	4–6 months	Starting point for mildly low testosterone
Moderate hypogonadism (T 200–300)	800–1000mg T	4–5 months	Most common male dosing range
Significant deficiency (T <200)	1000–1200mg T	4–5 months	Higher dose – monitor hematocrit closely at 4–6 weeks
Very active / high BMI	1000–1400mg T	4–5 months	Higher clearance – may need higher dose or shorter interval
Note	Estrogen management in males	–	If E2 rises >40 pg/mL: consider anastrozole 0.5mg 2x/week or reduce T dose at re-pellet

Insertion Technique – Key Steps	
Site	Upper outer buttock — gluteal fat pad. Lateral to gluteal crease, medial to iliac crest. Mark site with pen.
Prep	Betadine and alcohol prep. Sterile field. Lidocaine 1-2% with epinephrine — infiltrate skin and tract to fascia.
Incision	2–3mm incision through skin only with a #11 scalpel. Do not go too deep.
Trocar insertion	Insert trocar through incision into the subcutaneous fat layer parallel to fascia. Advance toward iliac crest.
Pellet delivery	Load pellet into trocar. Advance stylet to deliver pellet. Withdraw trocar while holding stylet in place.
Closure	Steri-strips or Dermabond to close incision. No sutures needed. Apply pressure 60 seconds.
Post-care	No soaking (baths/pools/hot tubs) for 5–7 days. No strenuous lower body exercise 3–5 days. Bruising normal.

Complications – Recognition & Management		
Complication	Signs	Management
Infection	Redness, warmth, purulent drainage, fever	Antibiotics (Keflex 500mg QID x 7–10 days). Rarely requires pellet removal
Extrusion	Pellet visible under skin, migrating toward incision	Apply pressure dressing. Rarely extruded fully — monitor closely
Fibrosis / scarring	Hard nodule at insertion site on subsequent insertions	Rotate insertion sites. Massage area post-insertion to prevent
Bruising / hematoma	Significant swelling or bruising at site	Ice, pressure, NSAIDs if no contraindication. Resolves 1–2 weeks
High testosterone (women)	Acne, oily skin, clitoral changes, hair growth	Cannot remove pellet — manage side effects pharmacologically until dissolves
High estradiol (men)	Gynecomastia, water retention, mood changes	Anastrozole 0.5mg 2x/week. Lower T dose at next pellet

Monitoring Post-Pellet		
Timepoint	Labs to Draw	What to Assess
4–6 weeks post-pellet	Total T, free T, E2, hematocrit (men), SHBG	Peak pellet effect. Adjust next dose based on these levels + symptom response
Pre-pellet (end of cycle)	Same panel	Trough levels. Shows how far levels dropped — informs re-pellet timing and next dose
Every re-pellet	Full panel per protocol	Trending response. Adjust dose if under or over responding
Annual	Full hormone panel + standard screening labs	PSA (men), mammogram (women), CBC, metabolic panel